

The Global Burden of Mental Illness

Prevalence, impact, and the persistent treatment gap

Our World in Data / IHME

Public health policy + planning



DALYs
Prevalence
Care Gap

Hundreds of millions are affected each year

Mental health conditions affect well-being, the ability to work, and relationships with friends, family, and community.

1 in 3

women will experience major depression in their lifetime

1 in 5

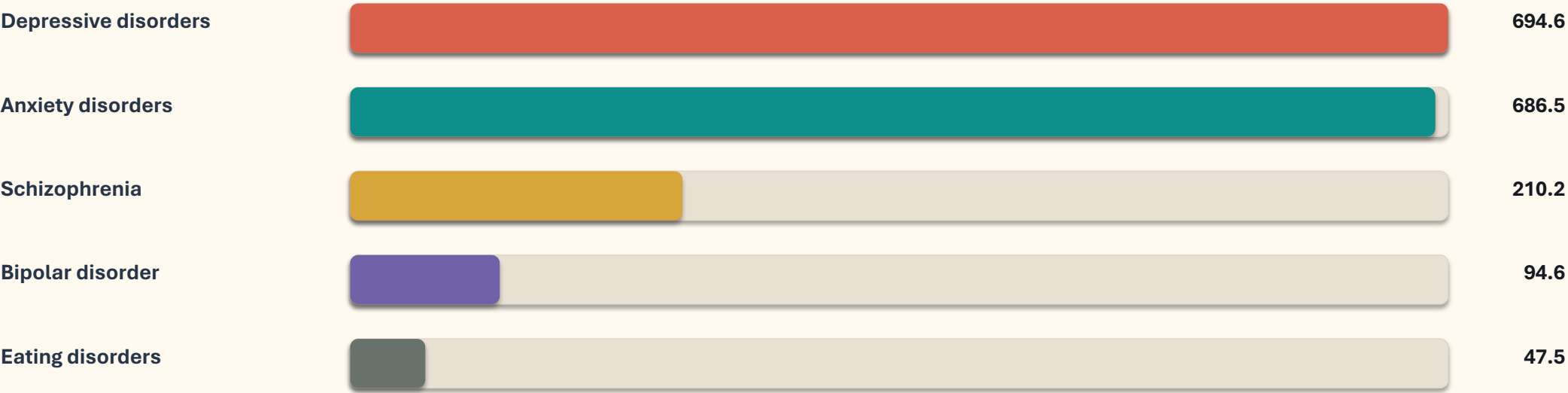
men will experience major depression in their lifetime

Conditions span common depression and anxiety.

Schizophrenia and bipolar disorder are less common but have a large impact on individuals' lives.

This scale makes mental health a core policy issue.

Depression and anxiety dominate measured DALY burden



DALYs per 100,000 people

DALYs combine years of life lost due to premature death and years lived with disability.

Figure reference: burden of disease from each category of mental illness, World, 2023

NEAR PARITY IN GLOBAL BURDEN

Depression and anxiety carry nearly equal disease burden v

Together, they account for roughly 80% of the total mental health disease burden across these five categories.

694.6

Depressive disorders

686.5

Anxiety disorders

Difference: 8.1 DALYs per 100,000

The core planning implication is not choosing between depression and an

Nearly identical DALY rates

LOWER PREVALENCE, HIGH INDIVIDUAL IMPACT

Schizophrenia and bipolar disorder require sustained spec

Depressive disorders



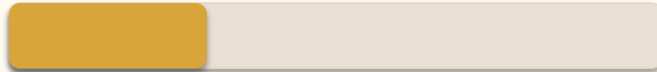
694.6

Anxiety disorders



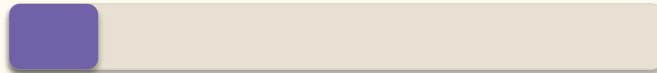
686.5

Schizophrenia



210.2

Bipolar disorder



94.6

DALYs per 100,000 people

Why this still matters

Schizophrenia contributes 210.2 DALYs per 100,000.

This reflects severe disability associated with the condition.

Bipolar disorder contributes 94.6 DALYs per 100,000.

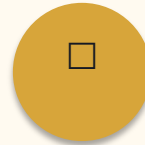
Treatment exists, but care is often lacking or poor quality



Treatable



Access



Quality



Stigma

Mental illnesses are treatable and their impact is often lacking, especially in low- and middle-income countries. Even where treatment exists, it is often of poor quality. Disclosure of psychiatric symptoms suppresses help-seeking and prevalence estimates.

Underreporting means the actual burden may be higher than reported figures suggest.

People respond through medication, faith, and social support

Global survey data reveals wide variation in how people deal with anxiety or depression.



Prescribed medication

Common in higher-income settings.



Religious or spiritual activities

Prevalent in many regions worldwide.



Friends or family

A widely reported coping mechanism.



...y of coping strategies underscores that policy interventions must account for cultural, economic, and healthcare system factors rather than applying a one-size-fits-all

Prioritize depression, anxiety, and closing the treatment gap

1

Depression and anxiety are the dominant contributors to the global mental health burden.

2

Schizophrenia and bipolar disorder carry disproportionate individual burden and high costs.

3

Treatment exists, but access and quality remain inadequate in many settings.

4

Stigma suppresses help-seeking and makes true prevalence difficult to estimate.

5

Scale evidence-based interventions for common mental disorders while addressing barriers to care.

→

Good data is essential to understand how conditions occur and how they can be better managed.